

Gan Clinical Letters: 10-Record Reading Packet

Clinical letter text only; annotations and labels omitted.

Letter 1

Department of Neurology

Clinic Date: 02 October 2025

Dr Eleanor Matthews

St Bartholomew's Hospital

Department of Neurology, West Smithfield, London EC1A 7BE

Dear Dr Eleanor Matthews,

Michael Turner, DOB: 14-03-1989, Hospital No: H392745 NHS No. 9034512786

Flat 3, Willow Supported Living, 22 Brook Lane, London, SE15 4JG

Diagnoses:

Generalised epilepsy with documented generalised spike-and-wave activity on prior EEG (2017)

Current Medication:

Sodium valproate 500 mg bd

Clobazam 10 mg nocte (as rescue on cluster days)

Plan:

- Continue sodium valproate 500 mg twice daily
- Maintain clobazam 10 mg at night for up to 3 nights during cluster periods, as per existing protocol
- Continue seizure charting by supported accommodation staff and share monthly summaries
- Consider ambulatory EEG if frequency or pattern worsens
- Safety advice reinforced; SUDEP risk discussed; emphasis on sleep regularity and alcohol avoidance

I reviewed Michael Turner in my clinic today. He lives in supported accommodation where rotating staff maintain daily seizure charts; these charts, which I reviewed, are used as our primary source of frequency data. Over the last four weeks, the records note: "Cluster days twice this month; typically six seizures in 24 h." These events are brief generalised convulsions with rapid recovery, no new focal features, and no injuries reported. Staff have been consistent in logging times, witnessed features, and rescue medication use.

He describes a prodrome of diffuse head pressure and photophobia on days preceding clusters, occasionally with poor sleep the night before. There is no clear relation to intercurrent illness, but stress and missed meals were mentioned as possible triggers. He reports good adherence to sodium valproate, and staff confirm administration records align. Where rescue has been needed, clobazam 10 mg nocte has been used for up to three consecutive nights with good effect and without excessive daytime sedation.

Neurologically he is unchanged on examination. Weight stable, no tremor, and no rash. Liver function tests in August were within normal limits; we will repeat monitoring at the usual interval. He remains seizure-free between clusters for prolonged stretches, and there have been no emergency department attendances this year.

We discussed options. Given the current balance of efficacy and tolerability, I recommend continuing sodium valproate at the present dose, with staff continuing meticulous charting. If clusters increase in frequency or if daytime sedation from clobazam becomes problematic, we can consider either a small valproate titration or introducing lamotrigine as adjunct. For now, no immediate changes are required.

I have provided updated written guidance for staff regarding when to initiate clobazam on cluster days, thresholds for calling 999, and parameters for documenting duration and post-ictal recovery (- please file in the medication folder). Standard safety advice was reiterated, including supervision when bathing, avoiding heights, and maintaining sleep hygiene. We also reviewed SUDEP information briefly and gave a leaflet for the team and key worker.

Follow-up: I will review him in six months, sooner if the pattern shifts (e.g. more than two cluster days per month or any prolonged seizure >5 minutes). The team will continue to email monthly seizure chart summaries to our clinic address.

With best wishes,

Yours sincerely

Letter 2

KINGS NEUROSCIENCES CENTRE

Clinic Date: 8 September 2010

Dr Patel

Elmwood Medical Centre

72 Elmwood Road, London, SE5 3HT

Dear Dr Patel

Jane Smith, DOB: 21-11-1982, Hospital No: H738291 NHS No. 0093384721

Flat 4, 21 Brook Lane, London, SE5 1AB

Thank you for asking me to review the above patient in the Neurology Clinic today. She is known to our service with focal epilepsy and, encouragingly, reports a steady improvement over recent months, which she attributes to better adherence following use of a consistent seizure diary and reminders on her phone.

Current treatment consists of Lamotrigine 150 mg b.d. and Levetiracetam 1 g b.d., both well tolerated without dose-limiting adverse effects. She reports improved energy levels compared to earlier in the year and no new intercurrent illnesses. There is no current concern regarding mood or sleep beyond occasional pre-examination anxiety, which she manages with standard breathing techniques taught by our epilepsy nurse specialist.

Regarding recent events: In February she experienced a prolonged focal seizure lasting 8 minutes, resolving without intervention. In May there were four further brief absences, and in August a single generalised tonic-clonic seizure was reported at school (not witnessed by family). These episodes have been clearly recorded in her diary, with triggers likely related to missed meals and sleep deprivation around coursework deadlines. Since adopting structured reminders in late August, she has had no further events and has taken all doses on time.

Neurological examination today is unremarkable. There are no drug-related rashes, no tremor, and no cognitive concerns on bedside screening. We discussed safety and rescue planning; no rescue medication has been required. She prefers to avoid changes while her control is improving, and I agree that stability is prudent given the recent positive trajectory.

Plan:

- Continue Lamotrigine 150 mg b.d. and Levetiracetam 1 g b.d. unchanged.
- Maintain seizure diary and phone reminders; continue avoidance of sleep deprivation and missed meals.
- Bloods today for routine monitoring (FBC, U&Es, LFTs) - to be copied to you.
- Review in clinic in 4 months, sooner if breakthrough seizures recur or if tolerability issues arise.

Overall, her focal epilepsy appears better controlled with improved adherence, and she is optimistic about maintaining this trajectory.

Yours sincerely

Letter 3

Epilepsy Centre

Neurosciences Division

From: Dr Thomas Reid (KING'S COLLEGE HOSPITAL NHS FOUNDATION TRUST)

Sent: 14 October 2019 10:15

To: neuro.epilepsy@nhs.net

Cc: Dr Meera Shah (KING'S COLLEGE HOSPITAL NHS FOUNDATION TRUST); Dr Patrick O'Neill (KING'S COLLEGE HOSPITAL NHS FOUNDATION TRUST)

Subject: Telephone review and travel-related considerations

Dear Thomas

Epilepsy Diagnosis:

Patient has generalised epilepsy.

Present Medication:

1. Sodium valproate 500 mg twice daily
2. Clobazam 10 mg at night as needed for jet-lag periods (patient-reported intermittent use)

Present Seizure Frequency:

Two events over the last five months.

History and Context:

This is a follow-up conducted by telephone as he is a frequent business traveller with irregular sleep and time zones. He reports adherence to valproate on most days but occasionally misses doses during long-haul flights. His first seizure occurred in May 2019 in Ireland, at night while asleep. He woke with rhythmic twitching of the right arm and a sense of déjà vu. The second event was in October 2019 in Scotland, also during sleep, lasting five minutes with a similar pattern of symptoms. He denies daytime collapses, injuries, or prolonged postictal confusion. No alcohol excess reported; significant sleep disruption around both events.

Plan of Action:

- Reinforced general measures for generalised epilepsy with emphasis on strict medication adherence when crossing time zones; advised use of alarms and written dosing plans for flights.
- Continue sodium valproate 500 mg twice daily. He prefers no change at present given infrequency of events and travel schedule.
- May use clobazam 10 mg at night for up to three nights around major time-zone shifts if sleep disruption is expected, as previously advised.
- Arrange routine EEG at next in-person visit when in London; if logistics difficult, consider ambulatory EEG coordinated around his travel.
- Safety counselling provided (no driving until compliant with DVLA guidance; avoid sleep deprivation, swimming alone, or climbing heights).
- Next review by telephone in three months or sooner if further seizures occur.

With best wishes,

Letter 4

King's College Hospital

Department of Neurosciences

Clinic Date: 15 September 2023

Dr A. Patel

Consultant Neurologist

King's College Hospital

Department of Neurosciences, Denmark Hill, London SE5 9RS

Dear Dr A. Patel

Consultant Neurologist,

James Carter, DOB: 14-03-1986, Hospital No: KCH472918 NHS No. 9473165820

Flat 12, 8 Millwright Lane, London, SE15 3AB

Thank you for your referral. I reviewed the patient in clinic following concerns about ongoing seizures in the context of his work with metal fabrication and intermittent arc-light exposure. He reports strict adherence to workplace safety practices, including a full protective welding visor and gloves, and says he does not remove the visor during active welding. He is otherwise in good general health. There is no recent history of head injury, meningitis, or alcohol misuse. Sleep has been variable due to shift changes at the workshop.

Regarding current events, he still has focal onset seizures four times per day, drop attacks occurring in batches, and tonic-clonic seizures 2 times per month, this long-standing pattern has persisted without major change. He and his partner describe the focal episodes as brief lapses with a rising internal sensation and impaired awareness lasting under a minute, often clustering late morning. The drop attacks tend to occur in short runs over two to three days, typically in the early evening after work, with quick recovery but occasional bruising. The tonic-clonic events are usually nocturnal or in the early hours; he is sore and fatigued for the following day. He denies clear photic triggers, and states the seizure frequency does not vary with welding versus non-welding days. He continues his regular medication regimen as previously prescribed and has not missed doses.

He does not drive and remains in manual employment with modified duties when fatigued. His employer is aware of his condition and has implemented buddy supervision during hot-work tasks. There have been no burns or occupational incidents attributable to events on the shop floor. He reports that hydration, regular breaks, and consistent sleep help him feel steadier, though not changing the overall frequency. No new neurological symptoms were volunteered. Examination in clinic today was unremarkable with normal speech, cranial nerves, tone, and coordination; gait steady.

I have advised ongoing documentation of events with dates, times, and context (workday versus rest day, sleep duration, and recent stressors), and asked him to bring his workplace risk assessment to the next appointment so that we can ensure appropriate safety planning remains in place. We also discussed continuing to avoid working at heights or near unguarded machinery during periods of increased events. He is agreeable to sharing seizure logs with your practice.

I will arrange routine follow-up and will update you if any new information emerges from collateral reports or monitoring. Please let me know if there are any concerns in the interim.

Yours sincerely

Letter 5

University College Hospital

National Hospital for Neurology and Neurosurgery

Clinic Date: 02 October 2025

Dr Amelia Rhodes

Riverside Medical Practice

14 Brook Street, London W1A 3QL

Dear Dr Amelia Rhodes,

Alex Morgan, DOB: 22-08-1991, Hospital No: U918273 NHS No. 3021187745

Flat 3, 7 Harrow Lane, London NW1 5TS

Summary/Diagnoses: Generalised epilepsy with photosensitivity and startle-triggered events; early-years childcare worker in a high-noise, multitasking environment; sleep fragmentation due to interrupted midday rest breaks; history of mild anxiety symptoms related to workplace interruptions.

Medication: Sodium valproate 500 mg twice daily.

I reviewed the patient in clinic today. They attended alone. They report ongoing challenges at work in a nursery setting where the ambient noise is high, tasks require frequent rapid switching of attention, and planned midday rest breaks are often interrupted by staffing needs. Over the past three months they describe several episodes per week of brief generalised events, typically characterised by abrupt behavioural arrest with eyelid fluttering and loss of awareness lasting 10-20 seconds, occasionally clustering in the early afternoon when rest has been curtailed. Colleagues have noted brief pauses and dropped tasks without warning, followed by rapid recovery. There have been two morning episodes with sudden generalised jerks on awakening, leading to the patient dropping lightweight items but without injury.

The patient has no focal aura or lateralising features. No tongue biting or sustained postictal confusion has been reported after the brief absence-type events. Sleep is curtailed by early starts and the unreliable rest period at midday. Alcohol is minimal, and there is no recreational drug use. They deny missed doses; a weekly pill organiser is used. There have been no recent infections or intercurrent illnesses.

Examination today was unremarkable with normal cardiovascular and neurological findings. Weight and blood pressure are within target range. Review of prior investigations: MRI brain (2023) was normal. EEG (2024) showed generalised 3 Hz spike-and-wave discharges provoked by hyperventilation and intermittent photic stimulation, consistent with generalised epilepsy. Routine bloods last month were normal including FBC and LFTs.

Seizures: Patient currently experiences several episodes per week of brief generalised absence-type seizures, with occasional morning myoclonic jerks on awakening. These episodes are more frequent on days when the midday rest break is interrupted.

Plan:

- Diagnosis reaffirmed as generalised epilepsy. Continue sodium valproate 500 mg twice daily for now; adherence is good and no adverse effects reported.
- Provide seizure diary template to document timing relative to rest break interruptions and high-noise periods at work.
- Workplace liaison letter (with patient consent) offering neutral information about generalised epilepsy and recommending predictable, uninterrupted short rest period where feasible, and avoidance of strobe-like lighting in play areas.
- Safety advice reiterated for childcare duties: ensure another adult supervises during water play or ladder use; avoid carrying hot liquids during early morning periods when myoclonic jerks have occurred.

- Consider dose optimisation or alternative broad-spectrum therapy if diary confirms persistent several episodes per week despite stable routine; review in 3 months with diary and, if needed, repeat EEG with sleep deprivation.

Yours sincerely,

Letter 6

Epilepsy Centre

Neurosciences Division

From: Dr Priya Shah (KING'S COLLEGE HOSPITAL NHS FOUNDATION TRUST)

Sent: 17 June 2020 10:12

To: kch.epilepsyclinic@nhs.net

Cc: Dr Michael Turner (KING'S COLLEGE HOSPITAL NHS FOUNDATION TRUST); Dr Emily Chen (KING'S COLLEGE HOSPITAL NHS FOUNDATION TRUST)

Subject: Review and medication adjustment

Dear Dr Shah,

Epilepsy diagnosis:

Focal epilepsy, likely temporal lobe onset, based on semiology and diary-linked patterns.

Present medication:

1. Lamotrigine 150 mg twice daily
2. Clobazam 10 mg at night as needed (intermittent use per patient)

Present seizure frequency:

According to the patient's seizure diary app, events have clustered around sudden noises; over the past six weeks there were four brief focal-aware episodes and one focal-impaired awareness spell, typically occurring when exposed to abrupt auditory shocks (e.g., dropped crockery or unexpected alarms). The diary time-stamps correlate with environments of heightened noise and suggest a pattern consistent with startle-triggered focal events rather than spontaneous occurrences.

Plan of action:

- We discussed optimising treatment for focal seizures with sensory triggers. The patient reports good adherence and stable lamotrigine serum levels previously. Given the diary-documented noise-precipitated events and occasional breakthrough focal-impaired awareness, I recommend adding lacosamide as adjunctive therapy while maintaining lamotrigine.
- Initiate lacosamide as follows: 50 mg at night for 1 week, then 50 mg twice daily for 1 week, then increase to 100 mg in the morning and 50 mg at night for 1 week, and thereafter 100 mg twice daily if tolerated. Continue this dose until review.
- Continue lamotrigine 150 mg twice daily unchanged for now. Clobazam may remain as intermittent rescue for anticipated high-risk noise exposures, but we advised minimising routine use to avoid sedation.
- The patient will continue to use the seizure diary app to log events, noting context (sound level, location, preceding stress, sleep quality) and any medication side effects. We asked them to enable the app's noise-trigger tagging feature to refine correlation analysis.
- Safety: advised on avoiding hazardous activities when episodes are more likely (e.g., handling hot liquids during known noisy times). No driving until seizure-free in accordance with DVLA guidance; we have reiterated current restrictions and the need to report any loss of awareness events.

Follow-up:

- Please arrange basic metabolic panel and ECG locally before up-titration to 100 mg twice daily to screen for any conduction concerns.

- I have scheduled a review in 4 months. If well-tolerated with improved control, we will consider cautious clobazam de-escalation. If breakthrough events persist despite lacosamide, we will discuss alternative adjuncts or video-EEG monitoring.

With best wishes,

Letter 7

King's College Hospital

Department of Neurosciences

Clinic Date: 02 October 2025

Dr A. Sharma

Consultant Neurologist

King's College Hospital

Department of Neurosciences, King's College Hospital, London SE5 9RS

Dear Dr A. Sharma

Consultant Neurologist,

Daniel Carter, DOB: 14-03-1986, Hospital No: K912345 NHS No. 9076543211

12 Brookfield Road, London, SE15 3QT

Thank you for your referral. I reviewed Daniel Carter in clinic today with his partner present. Unfortunately, there has been a clear deterioration over recent months, marked by an increase in seizure-related incidents and several episodes resulting in falls and minor injuries (forearm bruising and a superficial eyebrow laceration managed with steri-strips at a walk-in centre). He describes focal epilepsy with impaired awareness; events begin with a rising epigastric sensation and left arm tingling, followed by behavioural arrest, lip-smacking, and unresponsiveness for 60-90 seconds. Post-event confusion lasts around 20-30 minutes, often with headache and fatigue.

Frequency has escalated. He and his partner report that what were previously sporadic episodes have evolved into short bursts of activity linked to warm environments and sustained physical activity-particularly on school field afternoons and community fun-run days-after which he may have two to three events over a 24-48 hour period. Outside these activity-linked bursts, he is now averaging one focal impaired-awareness seizure every 7-10 days. No clear nocturnal convulsive events have been witnessed, though he wakes on some mornings with tongue bite indentations and myalgia suggestive of possible secondary generalisation overnight on two occasions in the last six weeks.

He has focal epilepsy. Onset was in 2019 following a mild traumatic head injury. MRI (2021) demonstrated right mesial temporal T2/FLAIR signal change without clear volume loss; EEG (2022) showed right temporal interictal sharp waves. Current anti-seizure medication: Lamotrigine 200 mg twice daily and Levetiracetam 1,000 mg twice daily, both reportedly taken reliably. He notes mood lability on Levetiracetam but prefers not to alter this while seizures are active. There is no recent alcohol excess; sleep has been fragmented due to childcare demands. No intercurrent infection reported.

Given the recent falls and injury risk, coupled with the apparent cluster pattern associated with warm, exertional community events, I am concerned about a worsening of seizure control. I have requested an urgent repeat MRI brain with epilepsy protocol and ambulatory EEG (72 hours) to capture both habitual focal events and any sleep-related activity. I have also issued a safety plan, including advice on hydration, gradual warm-up/cool-down for physical activity, and supervision during higher-risk periods, as discussed. He has agreed to track events using a diary and smartphone seizure app, including antecedents, duration, and recovery features.

I will review him in four weeks or sooner if further injuries occur. Depending on results, we may consider transitioning from Levetiracetam to Brivaracetam or adding Lacosamide, but I will defer a firm decision until investigations are complete. I have given interim written guidance to his workplace regarding temporary duty modification to reduce risk from ladders and hot environments. DVLA advice has been reiterated in line with current seizure activity.

Please let me know if there are any additional concerns or if further information emerges from primary care.

Yours sincerely

Letter 8

St Mary's Hospital

Institute of Neurology

Clinic Date: 02 October 2025

Dr Ahmed

St Mary's Hospital

Institute of Neurology, St Mary's Hospital, London W2 1NY

Dear Dr Ahmed,

Sophie Daniels, DOB: 12-05-1991, Hospital No: H948320 NHS No. 8934751206

14 Brookside Close, London, W10 6JP

Medication:

Levetiracetam 1000 mg twice daily. Patient reports tolerating this without clear adverse effects apart from occasional fatigue. No recent dose changes.

History since last review:

She describes intermittent nocturnal episodes over the past three months with post-event confusion on waking and occasional tongue-biting reported by her partner. She remains currently not driving by self-report. Clusters characterized by two focal impaired-awareness seizures; frequency unclear. She is keeping a basic diary but cannot specify how often these occur, as some events may be unwitnessed.

She notes increased stress at work and irregular sleep. No intercurrent illness, head injury, or new medications. Caffeine intake modest; no alcohol excess. Menses regular; no clear cyclical pattern noted by her. Family is supportive and monitoring overnight. No emergency department attendances.

Examination today was unremarkable with normal speech, gait, and cranial nerve examination. Blood pressure 118/72 mmHg, pulse 72 bpm regular.

Plan:

- Continue levetiracetam 1000 mg twice daily for now.
- Implement structured seizure and trigger diary with partner corroboration to help clarify timing and frequency of clusters and any precipitants (stress, sleep loss).
- Safety counselling reiterated, including bathing precautions and avoidance of heights or open water when alone. She remains off driving and understands to inform DVLA as appropriate per current guidance.
- Arrange routine EEG and updated serum biochemistry including U&E, LFT, and levetiracetam level (for reference only).
- Provide information on sleep hygiene and stress management resources; patient agreeable.
- We will review in clinic in 4 months or sooner if events escalate, any injury occurs, or if there is a change in pattern.

Yours sincerely

Letter 9

Epilepsy Centre

Neurosciences Division

From: Dr Emily Fraser (KING'S COLLEGE HOSPITAL NHS FOUNDATION TRUST)

Sent: 20 April 2023 10:15

To: epilepsy.centre@nhs.net

Cc: Dr Nikhil Shah (KING'S COLLEGE HOSPITAL NHS FOUNDATION TRUST); Dr Laura Bennett (KING'S COLLEGE HOSPITAL NHS FOUNDATION TRUST)

Subject: Review and medication adjustment

Hi Emily

Epilepsy Diagnosis:

Diagnosis remains under review with ongoing characterisation. No definitive electroclinical classification assigned at present.

Present Medication:

1. Sodium valproate 300 mg twice daily
2. Clobazam 5 mg at night

Present Seizure Frequency:

The parents state that since starting ketogenic diet he has had 4 drop attacks, the latest one on 17 Dec. They report no clear precipitating triggers; sleep is variable due to the family currently awaiting relocation from temporary accommodation, with crowded living conditions noted and frequent nocturnal disturbances.

Plan of Action:

- Continue ketogenic diet with dietician oversight; parents to maintain seizure and dietary logs and report any concerns regarding hydration or tolerance.
- Increase clobazam to 5 mg in the morning and 5 mg at night for two weeks, then 10 mg at night thereafter if daytime sedation occurs; maintain sodium valproate at current dose pending review of response and tolerability.
- Arrange ambulatory EEG within the next 6-8 weeks to assist with classification and to capture typical events if possible.
- Refer to hospital family liaison for support regarding housing-related stressors that may be impacting sleep and seizure threshold.
- Safety advice reiterated regarding drop attacks: use of protective headgear during high-risk activities and avoidance of unsupervised bathing.
- Follow-up in three months or sooner if attacks cluster or there is any injury.

With best wishes

Letter 10

Epilepsy Centre

Neurosciences Division

Clinic Date: 12 October 2019

Dr Patel

St Cuthbert's NHS Trust

Neurology Department, 12 Waverley Road, Bristol BS1 4QF

Dear Dr Patel,

Jordan Ellis, DOB: 14-11-1988, Hospital No: H392174 NHS No. 9081764521

Flat 3, 29 Brookfield Terrace, Bristol BS7 1NL

Diagnoses:

Focal epilepsy, likely temporal lobe onset.

I reviewed Jordan Ellis in the epilepsy clinic today. They remain on long-term antiseizure therapy, with bone-health monitoring recorded elsewhere. They report stereotyped focal aware episodes characterised by a brief rising epigastric sensation and déjà vu, occasionally progressing to impaired awareness with automatisms lasting 1-2 minutes, followed by mild post-ictal fatigue. There have been no generalised tonic-clonic seizures reported. Seizure clustering is not described.

Since the last review, there has been no intercurrent illness, sleep pattern is stable, and there is no clear relation to menses or alcohol. Work attendance is good and no injuries have occurred. Importantly, the median inter-seizure interval approximately six weeks, with the patient noting that stress and missed meals can shorten this interval, while regular routines appear protective.

Neurological examination today was non-focal. No new adverse effects from medication were reported. Driving status and SUDEP risk were discussed; no red-flag features such as prolonged impaired awareness, cyanosis, or trauma were reported. Emergency care plans remain unchanged as seizures self-terminate within minutes and there has been no requirement for rescue medication.

We agreed to continue current management and to keep a detailed seizure diary capturing potential precipitants, duration, and level of awareness during events. Investigations and bone-health reviews are being coordinated through existing pathways and are documented elsewhere. We will plan routine follow-up in six months, or earlier should there be a change in frequency, semiology, or recovery profile.

Yours sincerely